

ernie

A daily visit that notices a bad day at home before it becomes a hospital day.

Christopher Pluntke · September 2026



Margaret is 84.

Every morning she signs Ernie's book. This morning she was slower, and nobody was there to notice.

Hypoactive delirium, the most common kind, looks like nothing at all: quieter, slower, a little withdrawn. The cause is usually cheap to fix on the day: an infection, dehydration, a new tablet. Missed, it ends in the emergency room.

60–75%

of cases missed, even in hospital

7 M

hospital episodes a year in the US

Delirium is a change from your own baseline. The home never had one.

[Start again](#)

Step 1 of 4: Signing in

**Thank you,
Margaret.**

✓ **Done**

Lovely to see you. Here is your page in the book.

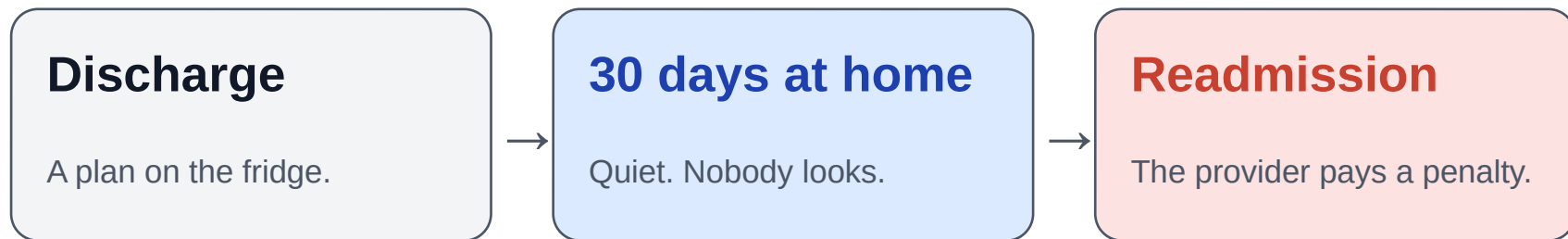


Thursday 17 September

Carry on

The gap is the 30 days at home.

Value-based care makes providers carry the risk for outcomes. The outcome is decided at home, between visits, where nobody looks.



\$36 B

a year in readmissions

2.0 M

readmissions a year

up to 3%

of Medicare payments at risk

\$8 k

extra per delirium episode



No tests. Just a visit.

She makes something every day: a signature, her own words, a finished picture. Each one is also a measurement.

Start again

Step 1 of 4: Signing in

Good morning, Margaret.

Sign my book to come in.



Any signature is fine. There is no wrong way.

Start again

Done signing

Sign the book

motor baseline · orientation

Start again

Step 2 of 4: A few questions

What day of the week is it today?

Sunday

Monday

Tuesday

Wednesday

Thursday

A few questions

her own words · latency

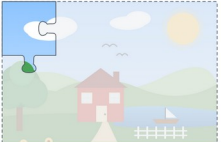
Start again

Step 3 of 4: A puzzle

Finish the picture

Drag a piece onto the picture.
Anna will help.

11 pieces left · Anna is here



Show me where this goes

Leave it for now

A puzzle, with Anna

attention · space · company

Start again

Step 4 of 4: Goodbye

Goodbye, Margaret.

✓ **Done**
That is everything for today.

Thank you for your company.
I will be here again tomorrow morning.

Start again

Goodbye

did she come · how fast

What the phone saw.

“Margaret’s signature was slower, more hesitant and a different shape than usual today.”



Her usual, and today. It looks the same. It wasn't.

What the family sees. Margaret sees a thank-you and her page in the book.

FEATURE	TODAY	USUAL	ROBUST Z
Time to first stroke	418 ms	420 ms	-0.1
Total time	4.46 s	3.06 s	+5.7 unusual
In-air time	1.07 s	267 ms	+37.8 unusual
Strokes	3	3	+0.0
Mean speed	0.650	0.778	-2.1 unusual

Nine features against her own five-day baseline: time to start, total time, pauses, strokes, speed, smoothness, size, shape.

Robust z: today minus her usual, over her usual spread. Well outside on two features is a flag. Three days is a trend.

Real numbers from the demo build.



Five things, one visit.

Every bedside instrument checks the first four, once a shift, in hospital. None sees the fifth, because nobody comes back hours later.

Attention

Days backwards in the chat.
Search time between pieces in the puzzle.

Orientation

“What is today?”, asked the way a friend would.

Disorganised thinking

Coherence of her own words.
Order of placement in the puzzle.

Arousal

Did she open it. How long before the first stroke.

Fluctuation

Morning against evening. Today against her own baseline.

40% preventable

Water, sleep, glasses, hearing aids, a phone call. A flag has a cheap answer.

The same construct every day, at home, against her own usual. That is the white space.



The visit carries the clinic's questions.

A nurse writes the question in plain words. Ernie asks it every morning and reads the answer.

What does the scale say this morning?

Is it harder to breathe today than yesterday?

Can you say the days of the week backwards?

What have you been up to today?

The runbook refuses clinical words, instructions, and more than six questions a day.

When something changes, explain the loop

What changed

Her signature was slower and she paused longer.

What it usually means

A change in attention can be the first sign of an infection or a new tablet.

Who to call

The care line today. Not 911.

Designed, not yet built. Decision support, never a diagnosis; a human on every flag.

What's next

1

Data leaves the phone

One process, one file: events, baselines, a family link.

2

Three real people, three days

Does a finger signature hold a baseline? Does a live doodle beat a check-in?

3

The explanation layer

A flag opens what changed, what it means, who to call.

How it was built: nine design docs, a component kit that bakes in the 80+ criteria, a critic that audits every screen, and Claude for the research, for reading replies, and for the code.



Synthetic people only. Not a medical device.